

I: So first question, familiarity with AI. How familiar are you with AI technologies in general?

P: I'm not an expert.

P: But I have used AI in my job to help synthesize reports or come up with some lesson plans.

P: It's all about writing good prompts.

P: I know the school system is now using AI more and teaching kids responsible AI use.

P: And we've had some training on that.

P: I also know students who use it quite regularly when they're upset.

I: So the school is now teaching staff or also the children?

P: Both.

I: And the use that you've used it for, was that part of something the school approved, or did they tell you to use it, or was it your own choice?

P: Good question.

P: Um, actually both.

P: We have Copilot as part of our work software.

P: And then the federal government just came out with, hold on a second.

P: I have to look, it's called [GOVERNMENT AI TOOL]

I: [GOVERNMENT AI TOOL]?

P: Yes..

P: And that's one we have to use.

P: It's only for people who work in the federal government.

P: You have to use an ID card as a security measure.

P: Copilot is used through the school and through our email.

P: Our principal uses it to clean up emails.

P: We've had some training on that.

P: There's also a site, [AI TOOL]

P: I joined that because it helps interpret things and rewrite things and is supposed to make good graphs.

P: It's about \$15 a month, and I'm not sure I'll keep it.

P: Sometimes if I upload a PDF with scores, it reports them wrong.

P: It mixes things up or uses the wrong names.

P: So I don't use that as much.

P: Copilot didn't used to do bell curves, but now it does.

P: I've written prompts for that for my assessments so I can put visuals in my reports, because visuals help parents.

P: I used to use Google Gemini for that, but they stopped it unless you pay more.

P: So I've been trying to fine-tune what works for me.

I: So there have been recommendations from the school and also from the government to use specific tools, right?

P: Yes.

I: So has your organization introduced any AI tools or systems? so ,yes.

I: Did you disclose this use with any of your clients?

P: I've put it in my report that some of it has been used.

P: There's a standard phrase saying some of it was assisted by AI, and that I'm responsible for editing.

I: Okay.

I: For this question, we also want a score from one to ten for how familiar you think you are.

P: I'd say about a five or six.

I: Okay.

I: Then we'll move to the next question.

I: This is about diagnostics and assessment use.

I: What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think it has a role.

P: When I've done assessments, I can put scores in and ask it to make graphs.

P: For example, I had a rating scale with a parent, two teachers, and a student.

P: I asked it to make a graph so I could show how their scores compare.

P: I haven't asked it to analyze the scores or interpret what they mean.

P: I do that.

P: But once I've written my report, which can be 30 pages, I have to summarize it for another system.

P: I've asked AI to synthesize what I've written, and it does a good job.

P: I proof it, but that works.

P: I wouldn't trust it to interpret test results on its own.

I: That makes sense.

I: So is synthesizing just another word for summarizing?

P: Yes.

I: Okay.

I: Let me think.

I: So you do use it for some kind of diagnostics and assessment use.

I: Do you have any intake processes, or how does your day-to-day work look?

P: Like intake with a patient?

P: I don't, because I work in a school.

P: It's not like I have to have parent permission to work with a student.

P: I introduce myself to the student.

P: I generally know who they are because they're in my school.

P: That's why I'm not using AI with them in the room with me.

P: I've been to doctor's appointments where they ask, like, can I record this so they can synthesize and write up notes.

P: I don't do that.

P: Does that make sense?

P: I would have to have parent permission to record a student.

P: So I don't do that.

P: I don't know if other people do, but I don't.

I: Do you think that would be a possible benefit, or would you want to use that?

P: I don't know that I would.

P: There's confidentiality in meeting with a client or a student.

P: If you're recording it, then there's a record somewhere, and that can be subpoenaed for court.

P: I tend to be careful about what I put in writing.

P: I'll document that I saw a student and put a broad subject of what we talked about.

P: But I'm not writing clinical notes.

P: I can see where, if you're an adult and you consent in a therapy session, that might be different.

P: I've seen it used when I was a patient at a doctor's office, like for my [INJURY].

P: But I don't use it that way.

I: Yeah.

I: So you're saying it can be used in court, right?

I: Or they can subpoena you to release that information.

I: Is that regarding you, the client, or both?

P: Both.

P: When you work in a school, you work for a school district.

P: Some of the students I see are special education.

P: There's documentation of how many minutes I have to see them and what goals I'm working on.

P: I have to report progress on that.

P: Any of those documents can be subpoenaed if a parent feels I'm not providing the correct service.

P: Or any of my written notes.

P: I could be working with a student on IEP goals, and they might tell me they're gay or bi or something else.

P: If I had that written down, that could also be subpoenaed.

P: So I don't necessarily write all of that down.

P: I write what I need to document their progress on the goals.

P: Or if I'm working with a student who's suicidal and they kill themselves, the parents could subpoena all of my documents.

I: So in general, how do clients come to you?

I: You need parent permission, right?

I: But does the child decide they want to come to you, or is it the parent?

P: It's a little bit of both.

P: In high school, so ages 14 to 18, students will often come and request to see me.

P: I can see them one time on a topic without parent permission.

P: For example, if they broke up with a partner.

P: I can see them again on a different topic, like bullying.

P: But if it's the same topic twice, I have to get parent permission.

P: Usually it's just asking, can I see your child?

P: And then I'll share what the student is willing to share.

P: For elementary students, they're younger.

P: Sometimes they come and ask what I do and if they can talk to me.

P: Often it's part of their program because they're special education and need support.

P: It's about building rapport and explaining why they're there.

P: Then we work on what we need to work on.

I: Okay, that makes sense.

I: Okay, let's move on to the next question.

I: So this is about monitoring and progress tracking.

I: How do you feel about the role of AI in tracking patient progress over time?

P: I think it's really helpful in the fact that you can put in your prompt of what you want them to document or monitor and have them print out a graph or information.

P: I have to write progress reports every quarter, so four times a year, for a student on an IEP.

P: I can put in what the goal is, then type in observations over time and ask it to write something that includes all of that.

P: And then you proof it, of course.

P: That's been really helpful.

P: I just did an observation on a student for an hour and a half, and I was able to verbally speak into Copilot my observation with timestamps.

P: It did a time study of how much time they were on task and also wrote up a narrative of the assessment.

P: It took way less time than typing it all out and doing it myself.

P: So I feel it could be a really useful tool if you know the prompts that you need.

P: You also have to be sensitive to confidentiality.

P: I don't put the student's name in it at all.

P: I put the student's name in my own way so I can search and replace it later.

P: So there's the privacy piece.

I: Yeah.

I: What is IEP?

P: Individualized Education Plan.

P: If they're receiving special services because they have a learning disability or ADHD or whatever, that's what we call the services we give them.

P: And it's individualized to them.

I: Yeah.

I: And the Copilot use, that's during some form of assessment, right?

P: Yes.

I: So how are you speaking into it?

I: How did that go?

P: On our system, because we use Teams and Microsoft, if you hit Windows and H at the same time, a microphone comes up.

P: When Copilot is open, you just speak into it and tell it your prompt.

P: I was able to say, this is an observation.

P: Could you write it up with timestamps, put it in narrative form, do a task analysis of time on task based on the timestamps, and graph it?

I: So you made the observations yourself with timestamps and then gave that data to Copilot?

I: Or did you record the whole session and it chose the timestamps itself?

P: I went into the classroom and wrote all the information down with timestamps.

P: Then when I came back to my office, I turned on the microphone and read it out with the timestamps.

I: Okay.

I: And for prompting, what AI tool do you usually use?

P: Right now I'm using Copilot pretty much exclusively.

P: Google Gemini stopped making the graphs that I wanted.

I: Yeah.

I: So how would you feel about AI taking on monitoring tasks in your practice?

P: I think it has a good role if it's used well and ethically.

P: You have to be careful about privacy and confidentiality, who has access, and things like that.

P: In Copilot, conversations are saved, so who else can access them matters.

P: Everything has to be double or triple checked because it's not always accurate.

P: You can't assume it's correct and not read it over, because there are mistakes.

I: Yeah, so the main concerns are data privacy and correctness.

P: Yep.

I: Okay, then let's move on to the next question.

I: This is about administrative and practical support.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

I: You briefly mentioned that you already use it sometimes for reports or making graphs.

P: I think it does a good job with that.

P: I think it takes time to develop the prompts in order to get the information that you need.

P: If you don't give it the right prompts, you're not going to get good information.

P: So you really have to spend the time doing that.

P: What I like about Copilot is that it gives suggestions of what else it could do.

P: Like, do you want me to do this, or I could put it in this type of writing?

P: So if I'm asking it to summarize a report so I can put a summary somewhere else, I can say, as written by a school psychologist for [ORGANIZATION], in this format, please use these headings.

P: I've kind of honed it in.

P: And with the bell curves now, Copilot is like, okay, this is my preferred bell curve graph.

P: It knows my parameters now, so I don't have to retype everything every time.

P: I like that it kind of learns from you.

P: Before, I was printing out bell curves and drawing the lines myself.

P: Now I can have them color-coded with percentages and standard deviation bands.

P: It's really visual, and it takes less than two minutes for Copilot to do that.

I: So you said this has saved you quite some time, right?

P: Yes.

I: How much time do you think you're saving by using these tools?

P: I think I'm saving a couple of hours every week.

P: If I get better at it, it could probably save me more.

P: I just got the prompts for the bell curves because I had to change assessments.

P: One was over a year old and I couldn't use it anymore.

P: So I'm giving a new achievement assessment and had to write a new prompt.

P: But using it to make graphs inside reports saves a huge amount of time.

I: And you said prompting takes some time to get the hang of.

I: Did you get guidance from your employer, or was it mostly trial and error?

P: It's a bit of both.

P: Some of it is trial and error.

P: Some of it came from the [AI TOOL], which had built-in prompts.

P: We also have training every quarter.

P: There are AI prompt courses you can take about how to structure prompts and what information to include.

P: So it's a combination.

I: What concerns would you have about accuracy or liability?

I: We talked about accuracy already, but how about liability?

P: I think people could get lazy and rely on it too much without making sure the information or interpretation is correct.

P: If you use it to interpret test results, it might not be accurate.

P: Every report I write is a legal document and has to stand up in court.

P: So it matters where the information comes from.

P: Everything is written very carefully, like parent reported, student reported, progress reports showed.

P: You have to make sure all those steps are still there.

P: You also have to know where the information is going and consider privacy.

P: If I left my office unlocked, someone could access what's in that report.

P: Is that the kind of liability you mean?

I: Yeah, like who's at fault if something AI-generated is wrong.

P: It would be my fault regardless.

P: I'm the one responsible for making sure everything is correct.

I: Okay.

I: Let's move to the next question.

I: This is about psychoeducation and between-session tools.

I: What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I think there's a lot of good use for it.

P: I haven't played with that as much because I focus more on assessment use.

P: But depending on the therapy approach, you could ask it to create a CBT journal tailored to an elementary or secondary student.

P: You can make lesson plans or materials for students to refer to.

P: Our special education teachers have used it to make reminders for desks, like steps to calm down or where to go for support.

P: I think there's a lot of potential.

P: It depends on how you use it and whether you know the prompts.

I: How does psychoeducation or between-session support usually look now?

P: For example, with a high school student who gets frustrated and pulls her hair out, we talk about coping skills.

P: We discuss other ways she can cope and practice those skills.

P: We often write down steps together during the session.

P: Then we turn that into a chart she can keep in her binder or on her phone.

P: We also check in between sessions and communicate with staff.

P: Monitoring comes from other sources too, because students aren't always reliable reporters.

I: That sounds more like monitoring.

I: Do they also get homework or tasks between sessions?

P: Yes, some do.

P: They might document how often they felt a certain way and what coping strategy they used.

P: We practice coping techniques in session and then they try them out.

P: They document what they did, how long it took, and what worked best.

P: It depends on the student and their needs.

I: Yeah.

I: So, um, what if the AI provides information that contradicts your treatment approach?

P: Oh, good question.

P: I think everybody has their own biases.

P: So you have to look at whether it's your own bias, what the better information is, and review it to determine what's best for the client.

P: AI can produce a lot of things, but it doesn't know the client as well as you do.

P: It's really the clinician's interpretation of what's best for the client.

P: I think one of the downfalls of AI is that it doesn't know the person.

P: So you have to take it with a grain of salt and check it.

P: But also make sure you're not entrenched in your own view and unwilling to look at other options.

I: Would you worry about clients relying too much on AI between sessions?

P: Yes.

P: AI isn't a person, so it doesn't have a relationship with you.

P: I think students could get really connected to a bot.

P: There's so much interaction that isn't real life or with real people.

P: Students and people are unlearning how to interact with others in person.

P: You say things online or to a bot that you wouldn't say to a real person.

P: They don't really know you, so they give generalized replies.

P: They're logical, but not necessarily about feelings.

P: If you asked AI how to kill yourself and what you have in your house, AI might tell you what to do.

P: It wouldn't say, wait, why are you feeling that way?

P: Or that this might be temporary, or ask what other help you can get.

P: I can see people using it because it's more comfortable than talking to a real person.

P: That comfort could help some people.

P: But I don't think it knows when someone actually needs a real clinician or medication.

P: It's biased based on what the person tells AI and their perception.

P: It doesn't reason about the other side of the story.

I: Yeah.

I: So how would you monitor what the AI is telling your clients?

P: I ask them if they use it.

P: And what kinds of responses they're getting.

P: Sometimes there's a record on their phone, so we can review that.

P: I honestly haven't had many students I work with regularly who use it.

P: There's one student I'm meeting with soon who has used it.

P: I'm curious to see what it says.

P: Again, it comes down to perception and bias.

P: So I'm curious about it.

I: Okay.

I: Let's move on to the next question.

I: This is about risks and limitations.

I: We've mentioned a few already.

I: So what risks or concerns do you have about using AI in clinical work?

P: I think there's a lot of fear that AI will take over human jobs.

P: If a bot can do therapy cheaply or for free, is that the direction things will go?

P: My concern is that, like in the UK, anyone can call themselves a counselor.

P: They don't necessarily need a degree.

P: So if people are using AI prompts without being trained or certified, there's a risk of grooming or bad information.

P: There are a lot of bad actors out there.

P: So who's programming the AI?

P: Who's behind it?

P: Is it being used for good or for bad?

P: That's my question.

P: And who has access to the information?

P: Like with concerns about access to federal systems, who can get into that data?

P: Nothing is ever really gone from the internet.

P: Could things people say to AI be used against them later?

P: So again, what happens to the information and where does it go?

I: Yeah.

I: In the U.S., being a counselor or psychologist is a protected title, right?

P: Yes.

P: You need specific degrees and training.

P: In the UK, anyone can say they're a counselor.

P: There's also a lot of telehealth now.

P: Many students don't get services at [LOCATION], so they use telehealth outside the [LOCATION].

P: The clinician might not see things like skin picking or fidgeting under the table.

P: They don't always see the whole client.

P: A lot of nonverbal communication is missed.

P: That face-to-face interaction is important.

P: I worry that it's already going away because of social media.

I: Yep.

I: Um, have you thought about issues like bias?

P: Yeah, I think whoever is programming AI matters, and AI kind of learns, but it's also whoever is programming it.

P: So how are they trained and what is the bias there?

P: When I was in grad school, we had to think a lot about our own biases.

P: For example, if I'm a born-again Christian and don't believe in abortion or birth control, and a young woman comes in with issues related to that, am I the appropriate person to give a non-biased view or advice?

P: Or do I need to recuse myself and say I can't do this?

P: AI isn't going to do that.

P: You ask a question and it gives you an answer, but I don't know who programmed it or what philosophy they're using.

P: And it won't necessarily ask all the questions a therapist would to really know the client or show empathy.

I: Mm-hmm.

I: Yeah, makes sense.

I: So another question: what concerns do you have about transparency or explainability of AI decisions?

P: I have a lot.

P: There have been cases of students or teens talking about suicide and AI giving advice about ways to kill yourself.

P: It's bad enough that kids who cut or are anorexic can go online and find sites that tell them how to hide it or how to burn calories or cut so nobody sees.

P: So does AI just provide a bigger resource for people to use in ways that aren't helpful or therapeutic?

P: And then there's the fallacy of it.

P: When I work with a student, I get information from teachers and parents.

P: I have a fuller picture, not just the student's report.

P: AI wouldn't have that.

P: A student might not be an accurate reporter of what's going on.

P: Without outside information, it's really hard to treat a student properly.

I: Any other limitations you can think of now?

P: Those are what I can think of now.

I: Okay.

I: Let's move on to the next question.

I: This is about the client perspective and therapeutic alliance.

I: How do you think clients respond to AI being used in their treatment?

P: I think it depends on how you explain it to them and what parts you're using.

P: I wouldn't have them sit there and talk to Copilot about their problems and ask for solutions.

P: I'm not doing it in front of a client and I'm not recording them.

P: It's more like, here are some exercises or handouts you can use.

P: I could have made them myself, but it would have taken hours, and Copilot did it faster.

P: I'm not giving them anything I wouldn't have researched myself or proofed to make sure it's accurate and appropriate.

P: I think people can be lazy and not proof things, and then misinformation can happen.

P: It also depends on the prompt you give.

P: And prompts themselves can be biased based on your own biases.

I: For this one we'd also like a score from one to ten.

I: How willing do you think clients are for AI to be used in their treatment?

P: I think it depends on the client.

P: Teenagers and younger people under 25 or 30 would probably be around a seven or eight.

P: People over 40 or 50 would be much lower, maybe a three.

P: They're not as comfortable or knowledgeable about how it works.

P: And it depends on how it's being used.

P: I'm not recording sessions and letting Copilot generate what I say to clients.

P: I'm still using my own skills and knowledge.

P: So it depends on how much you're using it.

P: Does that answer your question?

I: Yeah.

I: So, um, you don't really see a future where you're actually recording the meetings you have, right?

P: I do not.

I: Do you have something like the therapeutic alliance?

P: What do you mean by that?

I: It's the alliance you have with your clients.

I: It's one-on-one, trying to figure out together how to get you to a better place.

I: How do you think that would be affected if you used AI during meetings or in general?

P: I think clients would wonder if you're really listening to them or if you're relying on somebody else to listen and come up with things.

P: The whole point is to sit face to face, have a conversation, build a relationship and rapport, and know when to ask probing questions or when not to.

P: Knowing when the person is ready, when to push or not push, and what questions to ask.

P: I don't know that AI could do that as well without knowing the person.

P: I don't know that it would improve trust.

P: The client might think, well, I can just do this on my own, so why am I seeing you?

P: In my case, it's free because it's paid by the school.

P: But in a clinical setting where someone is paying for therapy, they might think, why am I paying you when I could just do this myself?

P: That trust and rapport with the therapist really matter.

I: Yeah.

I: Do you think clients would be more or less engaged with AI-supported treatment?

P: Again, I think it depends on the client.

P: Some may prefer it.

P: There's a student I'm meeting with who's been using some AI tools.

P: They're worried about sharing things with their therapist because they're afraid of being judged.

P: They read into body language and facial expressions and think someone is disappointed or upset.

P: Some clients might prefer AI because you don't see facial expressions.

P: But others are fine with face-to-face and then supplementing it with resources, that I create myself.

P: I often give parents or students handouts or steps to calm down.

P: Now I can make those faster with Copilot, but it's the same information.

P: I don't think clients would be upset about that.

P: They would be more upset if I relied on Copilot to actually do the therapy or decide what to talk about.

I: So in general, that would mean most people would be less engaged?

I: But for some people who are uncomfortable sharing nonverbal signs, AI might feel safer?

I: Do you think it's better to use AI in those cases, or is face-to-face still better?

P: I think building the relationship and rapport is still better.

P: Eventually, clients can share things they're uncomfortable saying at first.

P: You can also use journaling, texting, or writing things down and sharing them.

P: We do that a lot with students and parents.

P: There are ways to handle that without AI.

P: I think people could get too comfortable with AI and lose the ability to relate to others.

P: And again, what information is AI giving them?

P: I still think having a person involved is better than relying on AI alone.

I: Okay.

I: For the last question, this is about professional identity, autonomy, and future conditions.

I: Which aspects of your work should remain strictly human?

I: And what would you need to feel confident about using AI safely in practice?

P: I think the actual one-on-one counseling or therapy session should remain human.

P: You can supplement with handouts, homework, or resources.

P: AI can help combine things into a clear handout.

P: Picking up the phone and talking to a parent is much better than sending an email or AI-scripted message.

P: The human touch goes a long way.

P: Hearing someone's voice, knowing if they're upset, and asking follow-up questions matter.

P: AI wouldn't necessarily pick up on that.

P: Having connections with teachers, parents, coaches, and others gives a well-rounded view of the student.

P: You can't get all of that through email.

P: You can't hear tone.

P: AI is similar in that way.

I: Yeah.

I: Do you have anything else to add?

P: No, I think your questions are good.

P: It will be interesting to see what your research shows.

P: AI and mental health are a big topic right now.

P: A lot of people are relying on it.